



C-DAC: Centre for Development of Advanced Computing
C-56/1, Sector-62, Noida - 201307, Uttar Pradesh,
91-0120-3063311-13.



Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	68 Yr/M
Sample Type	:	Ascitic	Lab No.	:	230112BF005
Referred Doctor	:		Patient Type	:	OPD
			Collection Date	:	12-Jan-2023 17:24
			Acceptance Date	:	12-Jan-2023 17:39
			Reporting Date	:	12-Jan-2023 19:40

Body Fluids Routine and Microscopy Examination

SPECIMEN Peritoneal Fluid

METHOD: Routine and Microscopy

<u>Test</u>	<u>Result</u>	<u>Unit</u>
PHYSICAL EXAMINATION		
Volume	30	ml
Colour	Pale yellow	
Appearance	Clear	
Coagulum	Absent	
Blood	Absent	
Deposit	Absent	
TOTAL LEUCOCYTE COUNT		
Total Leucocyte Count	60	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	15	%
Lymphocytes	85	%
Eosinophils		%
Monocytes		%
Basophils		%
Mesothelial cells	Present	Absent/ Present
Degenerated cells		Absent/ Present
Atypical cells		Absent/ Present
Others	RBCs are present.	Absent/ Present

Comments:

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 12-Jan-2023 17:24
Patient Name	: anonymous	Age/Sex	: 68 Yr/M	Acceptance Date : 12-Jan-2023 17:39
Sample Type	: Ascitic	Lab No.	: 230112BF005	Reporting Date : 12-Jan-2023 19:40
Referred Doctor	:	Patient Type	: OPD	

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 05-Jan-2023 09:30
Patient Name	: anonymous	Age/Sex	: 48 Yr/M	Acceptance Date : 05-Jan-2023 11:17
Sample Type	: Semen	Lab No.	: 230105SA001	Reporting Date : 05-Jan-2023 13:43
Referred Doctor	:	Patient Type	: OPD	

Semen Analysis

SPECIMEN: Semen

METHOD: Physical and Chemical Examination, Microscopy

Test	Result	Ref. Range and Unit
PHYSICAL EXAMINATION		
Time of Sample	11.05am	AM/PM
Time of Examination	12.00pm	AM/PM
Duration of abstinence	10	2 - 7 Days
Liquefaction Time at 37 Â° C	45	30 - 60 minutes
Volume	5	>1.5 ml
Appearance	hazy	
Colour	grey white	
Viscosity	viscous	Viscous
Reaction (pH)	7	7.2 - 7.8 .
MICROSCOPIC EXAMINATION		
Total Sperm Concentration	70	>15 Million/ml
Total Motility (PR + NP)	75	>40 %
Progressive Motility (PR %)	70	>32 %
Vitality	75	>58 %
Agglutination	present	Negative
Pus cells	0-1	Not Detected /hpf
Red Blood Cells	absent	Not Detected /hpf
Epithelial Cells	absent	Not Detected /hpf
CHEMICAL EXAMINATION		
Seminal Fructose	positive	Positive
MORPHOLOGY		
Normal morphology	70	>4.0 %
Abnormal morphology	30	%
(a) Head defects	60	%
(b) Neck Mid Piece	35	%
(c) Tail defects	05	%

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 05-Jan-2023 09:30
Patient Name	: anonymous	Age/Sex	: 48 Yr/M	Acceptance Date : 05-Jan-2023 11:17
Sample Type	: Semen	Lab No.	: 230105SA001	Reporting Date : 05-Jan-2023 13:43
Referred Doctor	:	Patient Type	: OPD	

Comments:

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-	
CR No	:	-	Sample No.	:	-	Collection Date : 10-Jan-2023 11:18
Patient Name	:	anonymous	Age/Sex	:	42 Yr/F	Acceptance Date : 10-Jan-2023 16:36
Sample Type	:	Urine	Lab No.	:	230110UD003	Reporting Date : 10-Jan-2023 17:28
Referred Doctor	:		Patient Type	:	IPD	

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 19-Jan-2023 11:47
Patient Name	: anonymous	Age/Sex	: 51 Yr/F	Acceptance Date : 19-Jan-2023 16:39
Sample Type	: Urine	Lab No.	: 230117UM001	Reporting Date : 20-Jan-2023 10:00
Referred Doctor	:	Patient Type	: IPD	

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 04-Jan-2023 14:15
Patient Name	: anonymous	Age/Sex	: 34 Yr/M	Acceptance Date : 04-Jan-2023 16:36
Sample Type	: Body Fluid	Lab No.	: 230104BF008	Reporting Date : 04-Jan-2023 18:30
Referred Doctor	:	Patient Type	: OPD	

Body Fluids Routine and Microscopy Examination

SPECIMEN Other

METHOD: Routine and Microscopy

<u>Test</u>	<u>Result</u>	<u>Unit</u>
PHYSICAL EXAMINATION		
Volume	1.0	ml
Colour	reddish	
Appearance	slightly turbid	
Coagulum	present	
Blood	present	
Deposit	absent	
TOTAL LEUCOCYTE COUNT		
Total Leucocyte Count	40	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	10	%
Lymphocytes	90	%
Eosinophils	00	%
Monocytes	00	%
Basophils	00	%
Mesothelial cells	absent	Absent/ Present
Degenerated cells	absent	Absent/ Present
Atypical cells	absent	Absent/ Present
Others	abundant RBCs present	Absent/ Present

Comments:

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-	
CR No	:	-	Sample No.	:	-	Collection Date : 04-Jan-2023 14:15
Patient Name	:	anonymous	Age/Sex	:	34 Yr/M	Acceptance Date : 04-Jan-2023 16:36
Sample Type	:	Body Fluid	Lab No.	:	230104BF008	Reporting Date : 04-Jan-2023 18:30
Referred Doctor	:		Patient Type	:	OPD	

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	22 Yr/F
Sample Type	:	Urine	Lab No.	:	230114UP002
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	14-Jan-2023 12:51
			Acceptance Date	:	14-Jan-2023 15:28
			Reporting Date	:	15-Jan-2023 19:47

Urine Protein

SPECIMEN: Urine **METHOD:** Strip Method/ Heat and acetic acid method/ Sulphosalicylic acid method.

<u>Test</u>	<u>Result</u>	<u>Ref. Range</u>	<u>Unit</u>
Protein	detected	Not detected	NA

Comments:

Urinary total proteins are negligible in healthy individuals. Levels are increased in diseases that impair renal function like Diabetes, Hypertension, Nephrotic syndrome and Drug nephrotoxicity.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-	
CR No	:	-	Sample No.	:	-	Collection Date : 10-Jan-2023 14:54
Patient Name	:	anonymous	Age/Sex	:	53 Yr/M	Acceptance Date : 10-Jan-2023 16:35
Sample Type	:	Urine	Lab No.	:	230110UD003	Reporting Date : 10-Jan-2023 17:28
Referred Doctor	:		Patient Type	:	OPD	

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	51 Yr/F
Sample Type	:	Urine	Lab No.	:	230119UD002
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	19-Jan-2023 11:47
			Acceptance Date	:	19-Jan-2023 16:39
			Reporting Date	:	20-Jan-2023 10:02

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 13-Jan-2023 13:53
Patient Name	: anonymous	Age/Sex	: 14 Yr/M	Acceptance Date : 13-Jan-2023 15:47
Sample Type	: Body Fluid	Lab No.	: 230113BF002	Reporting Date : 13-Jan-2023 20:15
Referred Doctor	:	Patient Type	: IPD	

Body Fluids Routine and Microscopy Examination

SPECIMEN Other

METHOD: Routine and Microscopy

<u>Test</u>	<u>Result</u>	<u>Unit</u>
PHYSICAL EXAMINATION		
Volume	1	ml
Colour	colourless	
Appearance	slightly turbid	
Coagulum	absent	
Blood	absent	
Deposit	absent	
TOTAL LEUCOCYTE COUNT		
Total Leucocyte Count	870	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	70	%
Lymphocytes	30	%
Eosinophils	00	%
Monocytes	00	%
Basophils	00	%
Mesothelial cells	absent	Absent/ Present
Degenerated cells	absent	Absent/ Present
Atypical cells	absent	Absent/ Present
Others	absent	Absent/ Present

Specimen : ET apirate.

Comments:

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 13-Jan-2023 13:53
Patient Name	: anonymous	Age/Sex	: 14 Yr/M	Acceptance Date : 13-Jan-2023 15:47
Sample Type	: Body Fluid	Lab No.	: 230113BF002	Reporting Date : 13-Jan-2023 20:15
Referred Doctor	:	Patient Type	: IPD	

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 19-Jan-2023 11:29
Patient Name	: anonymous	Age/Sex	: 37 Yr/F	Acceptance Date : 19-Jan-2023 16:40
Sample Type	: Urine	Lab No.	: 230119UM001	Reporting Date : 20-Jan-2023 10:00
Referred Doctor	:	Patient Type	: IPD	

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	14 Yr/M
Sample Type	:	Csf	Lab No.	:	230116CF001
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	16-Jan-2023 06:11
			Acceptance Date	:	16-Jan-2023 11:28
			Reporting Date	:	16-Jan-2023 13:35

CSF Routine and Microscopic Examination

SPECIMEN: CSF

METHOD: Routine and Microscopy

Test	Result	Unit
PHYSICAL EXAMINATION		
Volume	1.0	ml
Colour	colourless	
Appearance	clear	
Coagulum	absent	
Blood	absent	
Deposit	absent	
MICROSCOPIC EXAMINATION		
Total Leucocyte Count	2000	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	05	%
Lymphocytes	95	%
Eosinophils	00	%
Monocytes	00	%
Basophils	00	%
Degenerated cells	absent	Absent/ Present
Atypical cells		Absent/ Present
Others		Absent/ Present

Kindly send sample for cytopathological examination.

Comments:

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 16-Jan-2023 06:11
Patient Name	: anonymous	Age/Sex	: 14 Yr/M	Acceptance Date : 16-Jan-2023 11:28
Sample Type	: Csf	Lab No.	: 230116CF001	Reporting Date : 16-Jan-2023 13:35
Referred Doctor	:	Patient Type	: IPD	

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	28 Yr/F
Sample Type	:	Urine	Lab No.	:	230114UK001
Referred Doctor	:		Patient Type	:	OPD
			Collection Date	:	14-Jan-2023 09:36
			Acceptance Date	:	14-Jan-2023 15:24
			Reporting Date	:	15-Jan-2023 19:47

Urine Ketones

SPECIMEN: Urine

METHOD: Dipstick method/ Rothera's method

Test	Result	Ref. Range	Unit
Ketones	Not detected	Not detected	NA

Comments:

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 14-Jan-2023 09:50
Patient Name	: anonymous	Age/Sex	: 52 Yr/M	Acceptance Date : 15-Jan-2023 19:09
Sample Type	: Ascitic	Lab No.	: 230115BF002	Reporting Date : 15-Jan-2023 21:01
Referred Doctor	:	Patient Type	: IPD	

Body Fluids Routine and Microscopy Examination

SPECIMEN Peritoneal Fluid

METHOD: Routine and Microscopy

<u>Test</u>	<u>Result</u>	<u>Unit</u>
PHYSICAL EXAMINATION		
Volume	2	ml
Colour	yellow	
Appearance	clear	
Coagulum	absent	
Blood	absent	
Deposit	absent	
TOTAL LEUCOCYTE COUNT		
Total Leucocyte Count	33	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	10	%
Lymphocytes	90	%
Eosinophils	00	%
Monocytes	00	%
Basophils	00	%
Mesothelial cells	absent	Absent/ Present
Degenerated cells	absent	Absent/ Present
Atypical cells	absent	Absent/ Present
Others	occasional RBCs	Absent/ Present

Comments:

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 14-Jan-2023 09:50
Patient Name	: anonymous	Age/Sex	: 52 Yr/M	Acceptance Date : 15-Jan-2023 19:09
Sample Type	: Ascitic	Lab No.	: 230115BF002	Reporting Date : 15-Jan-2023 21:01
Referred Doctor	:	Patient Type	: IPD	

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-	
CR No	:	-	Sample No.	:	-	Collection Date : 06-Jan-2023 11:15
Patient Name	:	anonymous	Age/Sex	:	24 Yr/F	Acceptance Date : 06-Jan-2023 16:45
Sample Type	:	Urine	Lab No.	:	230106UD002	Reporting Date : 06-Jan-2023 17:47
Referred Doctor	:		Patient Type	:	IPD	

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	7 Yr/M
Sample Type	:	Bal	Lab No.	:	230111BF001
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	10-Jan-2023 19:25
			Acceptance Date	:	11-Jan-2023 00:42
			Reporting Date	:	11-Jan-2023 06:32

Body Fluids Routine and Microscopy Examination

SPECIMEN Bal

METHOD: Routine and Microscopy

<u>Test</u>	<u>Result</u>	<u>Unit</u>
PHYSICAL EXAMINATION		
Volume	01	ml
Colour	Colorless	
Appearance	Clear	
Coagulum	Absent	
Blood	Absent	
Deposit	Absent	
TOTAL LEUCOCYTE COUNT		
Total Leucocyte Count	50	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	90	%
Lymphocytes	10	%
Eosinophils	00	%
Monocytes	00	%
Basophils	00	%
Mesothelial cells	Absent	Absent/ Present
Degenerated cells	Absent	Absent/ Present
Atypical cells	Absent	Absent/ Present
Others	Few squamous cells and RBCs present	Absent/ Present

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 10-Jan-2023 19:25
Patient Name	: anonymous	Age/Sex	: 7 Yr/M	Acceptance Date : 11-Jan-2023 00:42
Sample Type	: Bal	Lab No.	: 230111BF001	Reporting Date : 11-Jan-2023 06:32
Referred Doctor	:	Patient Type	: IPD	

Comments:

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ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 10-Jan-2023 11:18
Patient Name	: anonymous	Age/Sex	: 42 Yr/F	Acceptance Date : 10-Jan-2023 16:36
Sample Type	: Urine	Lab No.	: 230110UM001	Reporting Date : 10-Jan-2023 17:28
Referred Doctor	:	Patient Type	: IPD	

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	28 Yr/F
Sample Type	:	Urine	Lab No.	:	221228US001
Referred Doctor	:		Patient Type	:	OPD
			Collection Date	:	14-Jan-2023 09:36
			Acceptance Date	:	14-Jan-2023 15:24
			Reporting Date	:	15-Jan-2023 19:47

Urine Sugar

SPECIMEN: Urine

METHOD: Dipstick Method/ Benedict's Method

Test	Result	Ref. Range	Unit
Urine Sugar	Not detected	Not detected	NA

Comments:

Urinary total proteins are negligible in healthy individuals. Levels are increased in diseases that impair renal function like Diabetes, Hypertension, Nephrotic syndrome and Drug nephrotoxicity.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 05-Jan-2023 12:01
Patient Name	: anonymous	Age/Sex	: 24 Yr/F	Acceptance Date : 05-Jan-2023 17:48
Sample Type	: Urine	Lab No.	: 221230UM004	Reporting Date : 05-Jan-2023 18:07
Referred Doctor	:	Patient Type	: IPD	

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-	
CR No	:	-	Sample No.	:	-	Collection Date : 11-Jan-2023 11:19
Patient Name	:	anonymous	Age/Sex	:	42 Yr/F	Acceptance Date : 11-Jan-2023 16:32
Sample Type	:	Urine	Lab No.	:	230111UD002	Reporting Date : 12-Jan-2023 17:41
Referred Doctor	:		Patient Type	:	IPD	

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 03-Jan-2023 09:11
Patient Name	: anonymous	Age/Sex	: 47 Yr/M	Acceptance Date : 03-Jan-2023 11:24
Sample Type	: Semen	Lab No.	: 230103SA003	Reporting Date : 03-Jan-2023 13:14
Referred Doctor	:	Patient Type	: OPD	

Semen Analysis

SPECIMEN: Semen

METHOD: Physical and Chemical Examination, Microscopy

Test	Result	Ref. Range and Unit
PHYSICAL EXAMINATION		
Time of Sample	11.10am	AM/PM
Time of Examination	12.10pm	AM/PM
Duration of abstinence	15	2 - 7 Days
Liquefaction Time at 37 Â° C	45	30 - 60 minutes
Volume	2.5	>1.5 ml
Appearance	hazy	
Colour	grey white	
Viscosity	viscous	Viscous
Reaction (pH)	7.5	7.2 - 7.8 .
MICROSCOPIC EXAMINATION		
Total Sperm Concentration	60	>15 Million/ml
Total Motility (PR + NP)	<1	>40 %
Progressive Motility (PR %)	00	>32 %
Vitality	00	>58 %
Agglutination	present	Negative
Pus cells	absent	Not Detected /hpf
Red Blood Cells	absent	Not Detected /hpf
Epithelial Cells	absent	Not Detected /hpf
CHEMICAL EXAMINATION		
Seminal Fructose	positive	Positive
MORPHOLOGY		
Normal morphology	80	>4.0 %
Abnormal morphology	20	%
(a) Head defects	70	%
(b) Neck Mid Piece	25	%
(c) Tail defects	05	%

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 03-Jan-2023 09:11
Patient Name	: anonymous	Age/Sex	: 47 Yr/M	Acceptance Date : 03-Jan-2023 11:24
Sample Type	: Semen	Lab No.	: 230103SA003	Reporting Date : 03-Jan-2023 13:14
Referred Doctor	:	Patient Type	: OPD	

Comments: Kindly repeat sample after a proper counselling

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	37 Yr/F
Sample Type	:	Urine	Lab No.	:	230119UD004
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	19-Jan-2023 11:29
			Acceptance Date	:	19-Jan-2023 16:40
			Reporting Date	:	20-Jan-2023 10:02

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	8 Yr/M
Sample Type	:	Csf	Lab No.	:	230107CF001
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	07-Jan-2023 13:27
			Acceptance Date	:	07-Jan-2023 20:28
			Reporting Date	:	07-Jan-2023 22:47

CSF Routine and Microscopic Examination

SPECIMEN: CSF

METHOD: Routine and Microscopy

Test	Result	Unit
PHYSICAL EXAMINATION		
Volume	01	ml
Colour	colourless	
Appearance	clear	
Coagulum	absent	
Blood	absent	
Deposit	absent	
MICROSCOPIC EXAMINATION		
Total Leucocyte Count	50	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	05	%
Lymphocytes	95	%
Eosinophils	00	%
Monocytes	00	%
Basophils	00	%
Degenerated cells	absent	Absent/ Present
Atypical cells	absent	Absent/ Present
Others	occasional RBC seen	Absent/ Present

Comments:

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 07-Jan-2023 13:27
Patient Name	: anonymous	Age/Sex	: 8 Yr/M	Acceptance Date : 07-Jan-2023 20:28
Sample Type	: Csf	Lab No.	: 230107CF001	Reporting Date : 07-Jan-2023 22:47
Referred Doctor	:	Patient Type	: IPD	

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	42 Yr/F
Sample Type	:	Urine	Lab No.	:	230110UM002
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	11-Jan-2023 11:19
			Acceptance Date	:	11-Jan-2023 16:32
			Reporting Date	:	12-Jan-2023 17:43

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 17-Jan-2023 11:09
Patient Name	: anonymous	Age/Sex	: 51 Yr/F	Acceptance Date : 17-Jan-2023 16:28
Sample Type	: Urine	Lab No.	: 230116UD002	Reporting Date : 18-Jan-2023 10:26
Referred Doctor	:	Patient Type	: IPD	

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 17-Jan-2023 23:21
Patient Name	: anonymous	Age/Sex	: 14 Yr/F	Acceptance Date : 18-Jan-2023 01:05
Sample Type	: Csf	Lab No.	: 230118CF002	Reporting Date : 18-Jan-2023 03:14
Referred Doctor	:	Patient Type	: IPD	

CSF Routine and Microscopic Examination

SPECIMEN: CSF

METHOD: Routine and Microscopy

Test	Result	Unit
PHYSICAL EXAMINATION		
Volume	0.5	ml
Colour	colourless	
Appearance	clear	
Coagulum	absent	
Blood	absent	
Deposit	absent	
MICROSCOPIC EXAMINATION		
Total Leucocyte Count	00	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	00	%
Lymphocytes	00	%
Eosinophils	00	%
Monocytes	00	%
Basophils	00	%
Degenerated cells	absent	Absent/ Present
Atypical cells	absent	Absent/ Present
Others	absent	Absent/ Present

Comments:

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 17-Jan-2023 23:21
Patient Name	: anonymous	Age/Sex	: 14 Yr/F	Acceptance Date : 18-Jan-2023 01:05
Sample Type	: Csf	Lab No.	: 230118CF002	Reporting Date : 18-Jan-2023 03:14
Referred Doctor	:	Patient Type	: IPD	

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 06-Jan-2023 11:15
Patient Name	: anonymous	Age/Sex	: 24 Yr/F	Acceptance Date : 06-Jan-2023 16:45
Sample Type	: Urine	Lab No.	: 230106UM001	Reporting Date : 06-Jan-2023 17:50
Referred Doctor	:	Patient Type	: IPD	

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	51 Yr/F
Sample Type	:	Urine	Lab No.	:	230118UD004
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	18-Jan-2023 11:26
			Acceptance Date	:	18-Jan-2023 17:05
			Reporting Date	:	18-Jan-2023 19:15

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 06-Jan-2023 12:53
Patient Name	: anonymous	Age/Sex	: 50 Yr/M	Acceptance Date : 06-Jan-2023 16:36
Sample Type	: Pleural Fluid	Lab No.	: 230106BF003	Reporting Date : 06-Jan-2023 19:40
Referred Doctor	:	Patient Type	: IPD	

Body Fluids Routine and Microscopy Examination

SPECIMEN Pleural Fluid

METHOD: Routine and Microscopy

<u>Test</u>	<u>Result</u>	<u>Unit</u>
PHYSICAL EXAMINATION		
Volume	10	ml
Colour	Reddish	
Appearance	Slightly turbid	
Coagulum	Present	
Blood	Present	
Deposit	Absent	
TOTAL LEUCOCYTE COUNT		
Total Leucocyte Count	1000	Cells/mm ³
DIFFERENTIAL LEUCOCYTE COUNT		
Neutrophils	10	%
Lymphocytes	90	%
Eosinophils		%
Monocytes		%
Basophils		%
Mesothelial cells		Absent/ Present
Degenerated cells		Absent/ Present
Atypical cells		Absent/ Present
Others	Plenty of RBCs	Absent/ Present

Comments:

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-	
CR No	:	-	Sample No.	:	-	Collection Date : 06-Jan-2023 12:53
Patient Name	:	anonymous	Age/Sex	:	50 Yr/M	Acceptance Date : 06-Jan-2023 16:36
Sample Type	:	Pleural Fluid	Lab No.	:	230106BF003	Reporting Date : 06-Jan-2023 19:40
Referred Doctor	:		Patient Type	:	IPD	

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 17-Jan-2023 11:09
Patient Name	: anonymous	Age/Sex	: 51 Yr/F	Acceptance Date : 17-Jan-2023 16:28
Sample Type	: Urine	Lab No.	: 230117UM001	Reporting Date : 18-Jan-2023 10:26
Referred Doctor	:	Patient Type	: IPD	

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	31 Yr/M
Sample Type	:	Semen	Lab No.	:	230112SA002
Referred Doctor	:		Patient Type	:	OPD
			Collection Date	:	12-Jan-2023 11:35
			Acceptance Date	:	12-Jan-2023 12:01
			Reporting Date	:	12-Jan-2023 12:59

Semen Analysis

SPECIMEN: Semen

METHOD: Physical and Chemical Examination, Microscopy

Test	Result	Ref. Range and Unit
PHYSICAL EXAMINATION		
Time of Sample	11.50AM	AM/PM
Time of Examination	12.45PM	AM/PM
Duration of abstinence	3	2 - 7 Days
Liquefaction Time at 37 Â° C	35	30 - 60 minutes
Volume	2	>1.5 ml
Appearance	HAZY	
Colour	GREY WHITE	
Viscosity	VISCOUS	Viscous
Reaction (pH)	8	7.2 - 7.8
MICROSCOPIC EXAMINATION		
Total Sperm Concentration	10	>15 Million/ml
Total Motility (PR + NP)	40	>40 %
Progressive Motility (PR %)	20	>32 %
Vitality	40	>58 %
Agglutination	ABSENT	Negative
Pus cells	1-2	Not Detected /hpf
Red Blood Cells	ABSENT	Not Detected /hpf
Epithelial Cells	ABSENT	Not Detected /hpf
CHEMICAL EXAMINATION		
Seminal Fructose	POSITIVE	Positive
MORPHOLOGY		
Normal morphology	50	>4.0 %
Abnormal morphology	50	%
(a) Head defects	40	%
(b) Neck Mid Piece	40	%
(c) Tail defects	20	%

KINDLY REPEAT SAMPLE AFTER PROPER COUNCELLING

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 12-Jan-2023 11:35
Patient Name	: anonymous	Age/Sex	: 31 Yr/M	Acceptance Date : 12-Jan-2023 12:01
Sample Type	: Semen	Lab No.	: 230112SA002	Reporting Date : 12-Jan-2023 12:59
Referred Doctor	:	Patient Type	: OPD	

Comments:

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 13-Jan-2023 10:35
Patient Name	: anonymous	Age/Sex	: 34 Yr/M	Acceptance Date : 13-Jan-2023 11:27
Sample Type	: Semen	Lab No.	: 230113SA002	Reporting Date : 13-Jan-2023 14:22
Referred Doctor	:	Patient Type	: OPD	

Semen Analysis

SPECIMEN: Semen

METHOD: Physical and Chemical Examination, Microscopy

Test	Result	Ref. Range and Unit
PHYSICAL EXAMINATION		
Time of Sample	11:25	AM/PM
Time of Examination	12:15	AM/PM
Duration of abstinence	3	2 - 7 Days
Liquefaction Time at 37 Â° C	45	30 - 60 minutes
Volume	5	>1.5 ml
Appearance	Hazy	
Colour	Grey white	
Viscosity	Viscous	Viscous
Reaction (pH)	7.0	7.2 - 7.8
MICROSCOPIC EXAMINATION		
Total Sperm Concentration	60	>15 Million/ml
Total Motility (PR + NP)	60	>40 %
Progressive Motility (PR %)	40	>32 %
Vitality	60	>58 %
Agglutination	Absent	Negative
Pus cells	0-1	Not Detected /hpf
Red Blood Cells		Not Detected /hpf
Epithelial Cells		Not Detected /hpf
CHEMICAL EXAMINATION		
Seminal Fructose	Positive	Positive
MORPHOLOGY		
Normal morphology	60	>4.0 %
Abnormal morphology	40	%
(a) Head defects	50	%
(b) Neck Mid Piece	30	%
(c) Tail defects	20	%

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 13-Jan-2023 10:35
Patient Name	: anonymous	Age/Sex	: 34 Yr/M	Acceptance Date : 13-Jan-2023 11:27
Sample Type	: Semen	Lab No.	: 230113SA002	Reporting Date : 13-Jan-2023 14:22
Referred Doctor	:	Patient Type	: OPD	

Comments:

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Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 18-Jan-2023 11:26
Patient Name	: anonymous	Age/Sex	: 51 Yr/F	Acceptance Date : 18-Jan-2023 17:05
Sample Type	: Urine	Lab No.	: 230117UM001	Reporting Date : 18-Jan-2023 17:56
Referred Doctor	:	Patient Type	: IPD	

Urine For Myoglobin

SPECIMEN: Urine

METHOD: Qualitative, Chemical

Test	Result	Ref. Range	Unit
Myoglobin	Not detected	Not detected	NA

Note: Positive test should be correlated with clinical findings and immunoassay/ capillary electrophoresis.

Comments:

1. Myoglobin is a cytoplasmic protein of cardiac and skeletal muscle and passes into the circulation following damage to myocytes. It has an important role in Acute Myocardial Infarction, Early re-infarction and successful re-perfusion following therapy.
2. Myoglobinuria is associated with rhabdomyolysis and dermatomyositis.
3. As myoglobin is cleared by the kidney any change in GFR increases levels.

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Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	34 Yr/M
Sample Type	:	Semen	Lab No.	:	230217SA005
Referred Doctor	:		Patient Type	:	OPD
			Collection Date	:	17-Feb-2023 15:00
			Acceptance Date	:	17-Feb-2023 15:19
			Reporting Date	:	17-Feb-2023 17:28

Semen Analysis

SPECIMEN: Semen

METHOD: Physical and Chemical Examination, Microscopy

Test	Result	Ref. Range and Unit
PHYSICAL EXAMINATION		
Time of Sample	3:10 pm	AM/PM
Time of Examination	3:50 pm	AM/PM
Duration of abstinence	4	2 - 7 Days
Liquefaction Time at 37 Â° C	35	30 - 60 minutes
Volume	2	>1.5 ml
Appearance	hazy	
Colour	grey white	
Viscosity	viscous	Viscous
Reaction (pH)	7.5	7.2 - 7.8
MICROSCOPIC EXAMINATION		
Total Sperm Concentration	40	>15 Million/ml
Total Motility (PR + NP)	75	>40 %
Progressive Motility (PR %)	60	>32 %
Vitality	80	>58 %
Agglutination	present	Negative
Pus cells	occasional	Not Detected /hpf
Red Blood Cells	not detected	Not Detected /hpf
Epithelial Cells	occasional	Not Detected /hpf
CHEMICAL EXAMINATION		
Seminal Fructose	positive	Positive
MORPHOLOGY		
Normal morphology	70	>4.0 %
Abnormal morphology	30	%
(a) Head defects	50	%
(b) Neck Mid Piece	40	%
(c) Tail defects	10	%

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91-0120-3063311-13.



Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 17-Feb-2023 15:00
Patient Name	: anonymous	Age/Sex	: 34 Yr/M	Acceptance Date : 17-Feb-2023 15:19
Sample Type	: Semen	Lab No.	: 230217SA005	Reporting Date : 17-Feb-2023 17:28
Referred Doctor	:	Patient Type	: OPD	

Comments:

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	24 Yr/F
Sample Type	:	Urine	Lab No.	:	230105UD003
Referred Doctor	:		Patient Type	:	IPD
			Collection Date	:	05-Jan-2023 12:01
			Acceptance Date	:	05-Jan-2023 17:48
			Reporting Date	:	05-Jan-2023 18:07

Urine For Dysmorphic RBCs

SPECIMEN: Urine

METHOD: Microscopy

Test	Result	Ref. Range	Unit
Urine, Dysmorphic RBC	Negative	Negative	NA

Comments:

This test is used to localize the site of origin of hematuria. Red blood cells with cellular protrusions, membrane blebs, doughnut shapes and fragmentation are termed as dysmorphic RBCs. Presence of dysmorphic red cells is strongly suggestive of renal glomerular bleeding.

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	:	-	ABHA ADDRESS	:	-
CR No	:	-	Sample No.	:	-
Patient Name	:	anonymous	Age/Sex	:	29 Yr/M
Sample Type	:	Semen	Lab No.	:	230119SA003
Referred Doctor	:		Patient Type	:	OPD
			Collection Date	:	17-Jan-2023 09:27
			Acceptance Date	:	19-Jan-2023 14:19
			Reporting Date	:	30-Jan-2023 17:41

Semen Analysis

SPECIMEN: Semen

METHOD: Physical and Chemical Examination, Microscopy

Test	Result	Ref. Range and Unit
PHYSICAL EXAMINATION		
Time of Sample	10:15 AM	AM/PM
Time of Examination	...	AM/PM
Duration of abstinence	3	2 - 7 Days
Liquefaction Time at 37 Â° C	Non liquefaction of sample beyond 3 hours at 37degere centigrade	30 - 60 minutes
Volume	5	>1.5 ml
Appearance	Hazy	
Colour	Grey white	
Viscosity	Viscous	Viscous
Reaction (pH)	8	7.2 - 7.8
MICROSCOPIC EXAMINATION		
Total Sperm Concentration		>15 Million/ml
Total Motility (PR + NP)		>40 %
Progressive Motility (PR %)		>32 %
Vitality		>58 %
Agglutination		Negative
Pus cells		Not Detected /hpf
Red Blood Cells		Not Detected /hpf
Epithelial Cells		Not Detected /hpf
CHEMICAL EXAMINATION		
Seminal Fructose		Positive
MORPHOLOGY		
Normal morphology		>4.0 %
Abnormal morphology		%
(a) Head defects		%
(b) Neck Mid Piece		%
(c) Tail defects		%

Comments: Sample was collected twice for examination. However, microscopic examination and testing for fructose

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Department of Pathology And Lab Medicine

Clinical Pathology

ABHA NO	: -	ABHA ADDRESS	: -	
CR No	: -	Sample No.	: -	Collection Date : 17-Jan-2023 09:27
Patient Name	: anonymous	Age/Sex	: 29 Yr/M	Acceptance Date : 19-Jan-2023 14:19
Sample Type	: Semen	Lab No.	: 230119SA003	Reporting Date : 30-Jan-2023 17:41
Referred Doctor	:	Patient Type	: OPD	

could not be performed due to non liquefaction of the sample. Suggested further clinical and laboratory workup and management.

Note : High viscosity / non-liquefaction indicates male accessory gland malfunction.

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